

Post-migration stressors and mental health in SOGIE refugees: a critical review and conceptual model for quantitative measurement

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Abstract

Refugees with diverse sexual orientation, gender identity, and expression (SOGIE) experience alarming rates of mental health conditions, often exceeding those of non-SOGIE refugees and the general population. Despite this, quantitative research on this population remains extremely scarce, with less than a handful of studies looking at the impact of post-migration stressors. This critical review examines the existing literature to identify a methodological gap where current quantitative approaches fail to capture the structural and psychosocial hardships documented in qualitative research, such as retraumatizing asylum processes, intersectional discrimination, and exclusion from both home and host communities. To address this, the article proposes a comprehensive conceptual model integrating Minority Stress Theory with the taxonomy of Social Identity Threat. This model delineates both distal and proximal stressors mapped with SOGIE refugee experiences. It concludes by outlining measurable domains aligned with best practices for scale development, providing a foundational framework for future quantitative research to validate these stressors and improve culturally sensitive mental health care for SOGIE refugees.

Introduction

As of the end of June 2025, over 50 million people worldwide had fled their countries due to conflict, violence, and persecution, and were living in host countries as refugees (with granted status) or asylum seekers (with pending status; henceforth, “refugee” will refer to both groups for simplicity) (UNHCR, 2025). Although exact numbers are unavailable¹, individuals with diverse sexual orientation, gender identity and expression (SOGIE) face increased risk of persecution and forced migration. Same-sex activity was still criminalized in 33% of countries (66 of 194) as of 2024 (Kavanagh et al., 2025). While criminalization of gender identity and expression is harder to quantify², legislation targeting transgender and non-binary individuals has shown a clear and sharp global rise in 2025 (Restar et al., 2025).

The mental health of SOGIE refugees is both alarming and severely understudied. Most studies find that over 70% of SOGIE refugees have a mental health condition, including PTSD, anxiety, depression, and/or suicidal ideation³ (Bird et al., 2022; Bleckmann et al., 2025; Fox et al., 2020; Gottlieb et al., 2020; Hopkinson et al., 2017; Lasowski et al., 2023; Piwowarczyk et al., 2017). This prevalence is about five times higher than in the global population (World Health Organization, 2025), and exceeds that observed among both non-SOGIE refugees (Bird

¹ Data on the number of SOGI-based asylum claims is limited and variable across countries; e.g., ~13% of asylum cases in Canada between 2013-2015 were SOGI-based (Murugan et al., 2024), compared to ~2% in the UK in 2022 (Noralla, 2025).

² Officially, 13 countries had laws against cross-dressing or opposite-sex impersonation as of 2020, and 20 countries reported criminalizing transgender individuals as of 2023. Yet, systematic persecution of transgender people has been documented in several other nations (Kavanagh et al., 2025).

³ Rates of specific mental health disorders in SOGIE refugees reported in cited studies, rounded: Piwowarczyk et al., 76% rate of major depression – single or partial remission, 70% rate of PTSD, 28% rate of anxiety disorder, <10% rates for other mental health disorders screened (Piwowarczyk et al., 2017); Lasowski et al., 83% rate of PTSD, 73% rate of depression, 58% rate of anxiety (Lasowski et al., 2023); Gottlieb et al., 70% rate of anxiety and/or depression (Gottlieb et al., 2020); Fox et al., 80% rate of mental distress (instrument associated with depression, anxiety, or PTSD) (Fox et al., 2020); Bird et al., 79% rate of “probable” PTSD (Bird et al., 2022); Hopkinson et al., 72% rate of past or present suicidal ideation (rate for PTSD not reported) (Hopkinson et al., 2017); Bleckmann et al., specific rates not reported (Bleckmann et al., 2025).

et al., 2022; Gottlieb et al., 2020; Hopkinson et al., 2017) and SOGIE non-refugees (Bleckmann et al., 2025) in the same studies.

Despite the alarming rate of distress in SOGIE refugees, very few studies have quantitatively assessed mental health in this population, and even fewer have investigated the specific stressors leading to such mental health outcomes. To the best of my knowledge – and as reviewed in detail in a later section – there exist only two quantitative studies focused on understanding post-migration stressors (Bleckmann et al., 2025; Fox et al., 2020). The remaining, still limited, quantitative body of work focuses solely or predominantly on pre-migration persecution (Alessi et al., 2018; Bird et al., 2022; Hopkinson et al., 2017; Lasowski et al., 2023; Mattelin et al., 2022; Piwowarczyk et al., 2017). This paucity of quantitative assessments of stressors – especially those linked to post-migration experiences – creates a significant knowledge gap and limits our ability to address the mental health needs of SOGIE refugees.

In this critical review, I problematize the absence of quantitative research on post-migration stressors, and advocate for a comprehensive assessment of SOGIE refugees' mental health by proposing an integrative model based on reviewed lived experiences and theories. I begin by reviewing major literature to identify central themes of SOGIE refugee stressors, demonstrating the pervasiveness of post-migration factors, including asylum procedures, camp conditions, and psychosocial stressors in resettlement (Gottvall et al., 2023, 2025; Murugan et al., 2024; Nematy et al., 2023; Noralla, 2025; Yarwood et al., 2022). Through a critical examination of existing quantitative studies, I show that current quantitative approaches largely fail to incorporate post-migration experiences documented across qualitative literature (Alessi et al., 2018; Bird et al., 2022; Hopkinson et al., 2017; Lasowski et al., 2023; Mattelin et al., 2022; Piwowarczyk et al., 2017). I then examine how this methodological gap interacts with dominant narratives in media and policy to perpetuate an incomplete understanding of SOGIE refugees' stressors (Akin, 2017; Hiller, 2022; Jung, 2015; Murakami & Akilova, 2023; Peltonen & Jungar, 2018; Prearo, 2021; Raj, 2025; Tschalaer, 2020).

Finally, I propose a comprehensive conceptual model of SOGIE refugees' mental health. Integrating Minority Stress Theory (Meyer, 2003) with the taxonomy of Social Identity Threat (Branscombe et al., 1999), this model captures stressors across different stages of migration, expanding on both external and internal psychological facets. I conclude by delineating measurable domains aligned with Boateng's Best Practices for Developing and Validating Scales (Boateng et al., 2018). This work aims to provide a foundational tool for future quantitative research, addressing a pervasive gap and supporting a more comprehensive understanding of SOGIE refugees' mental health.

Themes encapsulating SOGIE refugees' stressors

Review of both qualitative and quantitative work reveals four broad themes encapsulating SOGIE refugees' stressors: 1) *Violence pre-migration, in transit, and in immigration facilities*; 2) *Biased and harmful asylum process practices*; 3) *Healthcare access and systemic obstacles in resettlement*; and 4) *Psychosocial challenges in resettlement and acculturation*. Furthermore, the analysis reveals a fifth thematic category that is not a direct stressor but provides critical context for the other themes: 5) *Media discourse reinforcing problematic narratives and stereotypes*.

It is important to note that this is not a comprehensive review of the extensive literature on SOGIE refugee stressors. Yet, the themes identified here align with and builds upon prior systematic or other reviews in SOGIE refugees (D'souza et al., 2022; Fournier et al., 2018; García Rodríguez, 2023; Gottvall et al., 2023, 2025; Murugan et al., 2024; Nematy et al., 2023; Noralla, 2025; Redcay et al., 2019; Selim et al., 2023; Yarwood et al., 2022).

Violence pre-migration, in transit, and in immigration facilities

Studies evaluating pre-migration factors contributing to mental health outcomes reveal that the majority of SOGIE refugees endure severe persecution pre-migration, including physical, sexual, and psychological abuse (Alessi et al., 2018; Bird et al., 2022; Lasowski et al., 2023), often beginning in childhood and perpetrated by family members (Hopkinson et al., 2017). Moreover, work demonstrates that there is a link between pre-migration experiences of violence and post-migration mental health outcomes (Bird et al., 2022; Hopkinson et al., 2017).

As reviewed by Yarwood et al., experiences of violence, harassment, and discrimination persist throughout the migration process, occurring both in transit, and in refugee camps or detention centers (Yarwood et al., 2022). The experiences of transgender individuals in immigration facilities have been documented as particularly traumatic, often involving abuse inflicted directly by immigration personnel (Marquez-Velarde et al., 2024).

Biased and harmful asylum process practices

SOGIE asylum procedures have been shown to be not only methodologically flawed – with adjudication decisions based on incorrect markers of credibility – but also retraumatizing and harmful for claimants (Danisi et al., 2020; Danisi & Ferreira, 2022; Dhoest, 2019; Gordon-Orr, 2021; Held & McCarthy, 2018; Selim et al., 2023). Refugees are expected to have openly had same-sex relationships, and they often confronted with sexually intrusive questioning and pressured to give explicit details about their sexual lives, with claims denied when responses are seen as too vague or modest (Selim et al., 2023).

Determination of asylum claims is overwhelmingly based on stereotyped views of what being SOGIE means, views that are based on western norms and do not truthfully apply to many SOGIE refugees (Grettrick, 2019; Shakhshari, 2020). For example, SOGIE refugees are expected to describe themselves using “LGBTQ” western-based labels (Selim, Helenelund, et al., 2025). Moreover, they are assumed to have attended Pride, lesbian bars or other stereotypically western spaces, and engagement in these events or places is often used as evidence to make a decision about the claim (Dhoest, 2019; Gordon-Orr, 2021; Powell, 2024; Selim et al., 2023). Additionally, refugees that reveal to be religious are often faced with comments that faith is at odds with holding a SOGIE identity, followed by intrusive questions and more difficult evidential requirements for obtaining asylum (Selim et al., 2023; Tschalaer, 2020). These narrow expectations can be even more inaccurate for less understood SOGIE individuals, such as bisexual, feminine lesbians, and black lesbians, leading to less successful asylum claims (Rehaag, 2008, 2009; Selim et al., 2023; Tschalaer, 2021).

Ultimately, success in the asylum process often requires SOGIE refugees to receive coaching on what to say and how to frame their experiences in ways that appear credible within Western perspectives (Casteleyn, 2024; Dhoest, 2019; Held & McCarthy, 2018; Selim et al., 2023). This is problematic not only because of inadequate access to resources that can provide this coaching, but also because of the psychological stress associated with concealing one’s true identity and performing behaviors that feel unauthentic and even uncomfortable (Casteleyn, 2024; Gordon-Orr, 2021; Selim et al., 2023).

Healthcare access and systemic obstacles in resettlement

Systemic barriers to healthcare access

Studies consistently report that SOGIE refugees face barriers to accessing mental health care, ranging from clinicians’ malpractice or mistreatment to systemic health services barriers (Alessi et al., 2023; Attia et al., 2022; Danisi et al., 2020; Yarwood et al., 2022) – as recently reviewed by Gottvall et al. (Gottvall et al., 2025).

Overt barriers are often faced in refugee camps or detention centers, including denial of treatment – in particular related to gender transition, but also HIV and depression – physical harm, sexual assault and overt verbal mistreatment by staff, including clinical personnel (Vogler & Rosales, 2023; Yarwood et al., 2022).

Less overt yet pervasive systemic barriers persist through resettlement. Language barriers are often reported (Danisi et al., 2020; Fox et al., 2020, 2024; Gottvall et al., 2024), aligning with broader immigration literature demonstrating the impact of communication anxiety on accessing mental health resources above all (Zhao et al., 2021). Cost and lack of appropriate documentation also pose challenges, especially at the asylum stage (notably, even some countries with universal healthcare do not offer free care to asylum seekers) (Attia et al., 2022; Danisi et al., 2020; Gottvall et al., 2024).

Lack of culturally-sensitive psychotherapy

Another barrier to healthcare access comes from mistrust following traumatic or otherwise negative experiences with clinicians, in refugee camps but also outside. Interviews with both SOGIE refugees and mental health practitioners highlight the widespread lack of cultural competence and resulting judgemental or insensitive behavior by practitioners (Alessi et al., 2023; Gottvall et al., 2024). This often results in SOGIE refugees struggling to find providers with whom they are comfortable. Interviews with clinicians reveal that, in the worst cases, SOGIE refugees experienced discrimination by their previous providers that lead to additional harm and difficulty within the therapy practice (Alessi et al., 2023). The general lack of mental health services that adequately account for the person's sociocultural context is a pervasive problem beyond SOGIE refugees (Ryder et al., 2019). There is great need for quantitative research analysing the cultural competence of psychotherapy resources currently available to SOGIE refugee and developing guidelines to improve care through cultural clinical psychology frameworks (Kirmayer & Ryder, 2016; Ryder et al., 2011).

Mental health stigma

Stigma and shame related to mental health can also contribute to not seeking care (Kahn et al., 2018). Different cultural background can be associated with different cultural models for what is normal, deviant, as well as stigmatized and judge negatively (Chentsova-Dutton & Ryder, 2020). As such, it has been suggested that culturally-sensitive interventions to promote mental health literacy together with improving cultural sensitivity of providers may improve utilization (Na et al., 2016). However, it is crucial not to generalize or assume that stigma is always the primary obstacle: discussing stigma in isolation may divert attention from structural challenges that should also be addressed. Indeed, when access is provided, such as in asylum shelter, SOGIE refugees have been found to utilize mental health resources at a significantly higher rate than their non-SOGIE counterparts, even when controlling for the rate of mental health need (Gottlieb et al., 2020). This suggests that the higher rate of mental health conditions in SOGIE refugees is not always due to an intrinsic reluctance to seek care.

Housing, employment and access to services

Systemic obstacles in resettlement span many aspects including housing, employment, language, and access services. As both SOGIE and refugee status are associated with heightened housing vulnerability and risk for homelessness, SOGIE refugees face intersectional challenges to finding housing, due to inadequate policies, discrimination, unaffordability (Danisi et al., 2020; McDowell & Collins, 2024; Wimark, 2021). This barrier is compounded by employment challenges resulting in financial hardship (Alessi, 2016; Danisi et al., 2020; Gottvall et al., 2025; Karimi, 2020; Nematy et al., 2023). This is because of policies limiting the legal right to work (e.g., asylum seekers may not be allowed to work before obtaining status or for a period), as well as qualifications not being recognized, lack of social connections, language barriers, and discrimination leading to lower income and job stability even after obtaining refugee status (Alessi, 2016; Danisi et al., 2020; Karimi, 2020). SOGIE refugees

also face access barriers to other types of services, such as legal, financial, or social, where language, discrimination and inadequate policies are some of the factors underlying these barriers (Danisi et al., 2020; Karimi, 2020; Murakami & Akilova, 2023; Ugurel Kamisli, 2021).

Psychosocial challenges in resettlement and acculturation

Intersectional discrimination

SOGIE refugees often face intersectional discrimination compounded by race and minoritized religious identities such as being Muslim (Alessi et al., 2020; Chossière, 2021; Danisi et al., 2020; Held, 2015; Noralla, 2025). This discrimination creates external barriers such as in housing, employment, healthcare and services access, and dating, as well as psychosocial challenges ranging from social exclusion to internal experiences of fear, shame and loneliness. For example, providers helping SOGIE refugees with housing have reported that prospective landlords or housemates frequently rejected tenants for being both “gay and Muslim”, and that even when housing was secured, many refugees endured anti-Muslim attitudes in their new home (Alessi et al., 2020). Often, SOGIE refugees face discrimination for being SOGIE from their home community or other refugees, and discrimination related to their country of origin or religion from the host country community, making it difficult to form social connections and leading to feelings of not-belonging and loneliness (Alessi et al., 2020). Discrimination at times escalates to intimidation, threats and harassment, leading to feelings of fear, unsafety, dread and not being free (Alessi et al., 2020).

Social connection and belonging

Alongside overtly negative comments and attitudes associated with discrimination, SOGIE refugees often face lack of understanding and of true acceptance by communities that share part, but not all, of their identities (Alessi et al., 2020; Held, 2023; Noralla, 2025). Local 2SLGBTQI+ spaces and community are often insensitive towards the diverse experience of SOGIE individuals from outside the Global North (Alessi et al., 2020; Held, 2023). For example, Black SOGIE refugees reported that local community members often draw attention to their race unsolicitedly and without context, including through remarks intended as compliments that nonetheless reinforce otherness (Chossière, 2021), skepticism about their SOGIE identity, bullying or exclusion from dating (Held, 2023). Likewise, refugees are often rejected by local 2SLGBTQI+ communities on the basis on their Muslim religious identity, at times being told that it makes them incompatible with queerness (Alessi et al., 2020; Held, 2023). Additionally, power imbalances between refugees and local individuals can lead to exploitation within 2SLGBTQI+ dating contexts (Alessi et al., 2020).

As intersectional experiences are not understood by either the home community or the host country 2SLGBTQI+ community, SOGIE refugees often feel like they do not belong in any space, leading to feelings of loneliness, alienation and loss, alongside complex identity negotiation (Alessi et al., 2020; Held, 2023; Noralla, 2025). Ultimately, when available, communities focused on 2SLGBTQI+ refugees or immigrants can often provide the kinship, solidarity and recognition not found in local 2SLGBTQI+ or home-country communities (Alessi et al., 2020).

Negotiating multiple identities

From asylum procedures to social circles, SOGIE refugees receive pressures to conform to unidimensional (e.g., Western) norms while abandoning key aspects of their original identities. This can lead to internal conflicts with identity negotiation and negative health outcomes, because successful cultural adaptation is characterized complex integration of heritage and mainstream cultures as well as other aspects of one’s identity, without abandoning one aspect of self in favor of another (M. Doucerain et al., 2013, 2017; Ryder et al., 2000).

Beyond the identities linked to heritage and mainstream culture, SOGIE refugees also have to navigate their SOGIE and refugee identities, and often also religious, ethnic, racial, and/or linguistic identities (Berry & Taban, 2021; Naseri, 2025). Work looking at the experiences of SOGIE refugees who are or were Muslim shows the complex journey of integrating these two identities (Alessi et al., 2021; Greatrick, 2023; Munir & Noor, 2023). Religious stigma toward one's SOGIE identity is often internalized as shame and self-blame and, together with the internal conflict perceived SOGIE and religious identities, it contributes to poorer mental health outcomes and suicidality (Alessi et al., 2021; Munir & Noor, 2023). While many refugees distance themselves from religion after resettlement, others integrate their SOGIE and Muslim identities, drawing resilience and strength from their faith throughout the migration and resettlement processes (Alessi et al., 2021).

While post-migration identity formation is important to wellbeing, broader immigration work shows that psychosocial adjustment, like securing housing, employment, physical and mental health, provides the necessary mental resources foundation to then support acculturation and identity negotiation (M. M. Doucerain et al., 2025). As such, cultural integration stressors should be considered together with systemic barriers to healthcare and services.

Media discourse reinforcing problematic narratives and stereotypes

Dominant narratives on forced migration portray the Global North as “morally superior” and much of the Global South as “inherently oppressive”, a narrative that critical scholars argue both simplistic and harmful (Akin, 2017; Greatrick, 2019; Hiller, 2022; Jung, 2015; Murakami & Akilova, 2023; Peltonen & Jungar, 2018; Prearo, 2021; Raj, 2025; Tschalaer, 2020).

First, critical scholars show that this narrative distorts reality, failing to capture complex truths. For example, only the Global North – which includes much of Europe, North America, and Oceania – is typically portrayed as the “progressive safe haven” for refugees (Murray, 2014; Schwanck, 2024). Yet, the vast majority of forcibly displaced individuals (over 85% as of a 2021 report) are hosted in the Global South, not the Global North (Murakami & Akilova, 2023). In contrast, the Global South – which includes several countries in Africa, South America, and part of Asia – is portrayed as having a culture that is inherently “unsafe” and “backward” (Akin, 2017; Holle et al., 2021; Murray, 2014; Schwanck, 2024). Yet, the Global North historically played a central role in establishing the very laws and practices in the Global South that are now condemned as oppressive (Franke, 2004; Holley, 2015). In particular, anti-SOGIE legislation is far from being inherent to native culture: rather, it was first introduced by European (Christian) colonization (Franke, 2004; Holley, 2015; Tschalaer, 2020). Post-colonial persistence of anti-SOGIE legislation is also suggested to be rooted in asymmetrical power relations with the Global North, reflecting an effort to reassert national freedom and independence (Franke, 2004; Tschalaer, 2020).

Second, these narratives juxtaposing Global North and Global South produce and reinforce stereotypes that are directly harmful to refugees, contributing to biased credibility assessments, discriminatory policies, and the erasure of agency (Alessi et al., 2025; Franke, 2004; Holle et al., 2021; Tschalaer, 2020). For example, these narratives invalidate the complex realities of SOGIE refugees whose experiences, as highlighted in the previous section, rarely align with simplistic rhetorics of purely “positive” post-migration “freedom.” By minimizing attention to the actual barriers and hardships refugees encounter, such narratives obstruct the development of effective solutions (Alessi et al., 2025; Jabr, 2021; Peltonen & Jungar, 2018). Moreover, moral narratives juxtaposing the Global South and North contribute to spreading openly racist and anti-Muslim or anti-Arab discourse, sentiments, and politics, hence increasing discrimination against many SOGIE individuals (Peltonen & Jungar, 2018; Tschalaer, 2020). Finally, cultural narratives impose Western-specific stereotypes of SOGIE

identity, which increase pressures for concealing parts of one's identity – like religion (Tschalaer, 2020). In particular, this narrative and stereotypes have been directly linked to the devastating asylum process discussed in the previous section, leading to invalidation of SOGIE refugee identities and frequent unfair asylum rejections (Akin, 2017; Alessi et al., 2025; Casteleyn, 2024; Dhoest, 2019; Gordon-Orr, 2021; Held & McCarthy, 2018; Murray, 2014; Tschalaer, 2020).

Despite the demonstrated consequences of these narratives, they remain deeply entrenched across multiple channels, including media, public discourse, humanitarian associations including 2SLGBTQI+ activism, governmental discourse and policies, and sometimes even academic research (Hiller, 2022; Jung, 2015; Schwanck, 2024; Tuzi & Carpi, 2024). Strikingly, the case of SOGIE refugees – particularly from Muslim-majority countries – is often instrumentalized to reinforce the prejudiced dichotomy between a "morally superior" West and an "oppressive" South (Dhoest, 2019; Hiller, 2022; Jabr, 2021; Jung, 2015; Schwanck, 2024; Tuzi & Carpi, 2024). This is done through a narrative that portrays SOGIE refugees as "vulnerable", "helpless" and "passive" victims that escaped homophobic cultures and are relieved and thankful to be rescued, while portraying the Global North as the "benevolent" and gay-friendly "saviour" that has compassion on these victims (Hiller, 2022; Peltonen & Jungar, 2018; Raj, 2025; Tschalaer, 2020).

Ultimately, emerging research prioritizing voices and perspectives directly narrated by SOGIE refugees themselves – without others' assumptions or agendas – offers the most powerful tool to genuinely understand their experiences and challenge inaccurate narratives (Capous-Desyllas et al., 2025; Capous-Desyllas & Akkouris, 2022; Marnell et al., 2021).

Critical gap in quantitative research on post-migration stressors

Identifying peer-reviewed studies with quantitative measures of mental health

The goal of the quantitative literature review was to identify peer-reviewed studies that include quantitative data on mental health outcomes, either by using standardized diagnostic or screening tools or via retrospective analyses of clinical charts and surveys. Searches were conducted on Google Scholar using combinations of search terms⁴ related to 1) refugees, asylum or forced migration, 2) mental health or stressors, and 3) SOGIE or 2SLGBTQI+ identity. A subsequent search iteration also included 4) terms related to quantitative methods or diagnostic or screening tools. Additional studies were identified through citation searching, and studies were then screened for the presence of quantitative measurement of mental health outcomes. Through this process, ten peer-reviewed studies were identified and are presented in Table 1 (Alessi et al., 2018; Bird et al., 2022; Bleckmann et al., 2025; Fox et al., 2020, 2024; Gottlieb et al., 2020; Hopkinson et al., 2017; Lasowski et al., 2023; Mattelin et al., 2022; Piwowarczyk et al., 2017).

While a full systematic review is beyond the scope of this article, the scarcity of studies identified here aligns with the results and consensus of recent comprehensive and systematic reviews and reports (D'souza et al., 2022; Noralla, 2025; Yarwood et al., 2022; Zardiashvili & Kasianczuk, 2019). For example, the recent 2025 report from the Rainbow Railroad organization identified only 14 quantitative and 12–13 mixed-methods studies on SOGIE refugees, even when including gray literature and empirical work not focused on mental health (Noralla, 2025). Similarly, reviews from D'Souza et al. (D'souza et al., 2022) and Yarwood et al. (Yarwood et al., 2022) each

⁴ Example search string (one iteration; modified iteratively during screening): (refugee OR refugees OR asylum OR "forced migration" OR "forced migrants" OR "forced displacement") ("mental health" OR psychosocial OR "psychological disorders" OR "psychiatric disorders" OR wellbeing OR stressors OR "risk factors" OR trauma OR traumatic OR PTSD OR depression OR anxiety) (SOGI OR "sexual orientation" OR "gender identity" OR LGB OR LGBT OR LGBTQ OR LGBTQ+ OR LGBTQIA OR LGBTQIA2S OR LGBTQIA2S+ OR lesbian OR gay OR queer OR homosexual OR bisexual OR asexual OR trans OR non-binary OR transgender OR transexual OR "sexual minority" OR "gender minority")

identified only three to four quantitative studies and a single mixed-methods study (the former examining peer-reviewed articles on SOGIE refugees' mental health, and the latter including both peer-reviewed and gray literature on health during transit). Yet other reviews have excluded quantitative studies (Gottvall et al., 2023, 2025; Nematy et al., 2023). Ultimately, the lack of quantitative data and research constitutes a critical research gap that is consistently identified by SOGIE organizations like Rainbow Railroad and Eurasian Coalition on Health, Rights, Gender and Sexual Diversity (Noralla, 2025; Zardiashvili & Kasianczuk, 2019).

Limited scope of stressors examined through quantitative approaches

Table 1 provides an overview of the methods used to measure mental health outcomes, stressors or determinants, and the associations between these variables across the ten identified quantitative studies.

When examining stressors, only three studies focused on post-migration factors to an extent at least comparable to pre-migration (Bleckmann et al., 2025; Fox et al., 2020; Gottlieb et al., 2020), and only two of these examined their association with mental health outcomes (Bleckmann et al., 2025; Fox et al., 2020). Two studies included some post-migration stressors but to a substantially lesser extent than pre-migration (Alessi et al., 2018; Lasowski et al., 2023), and one study assessed exposure to violence without specifying the phase, although the focus seemed primarily pre-migration (Mattelin et al., 2022); none of these three studies examined stressor–mental health associations. Three studies focused exclusively on pre-migration persecution factors and tested their associations with mental health outcomes (Bird et al., 2022; Hopkinson et al., 2017; Piwowarczyk et al., 2017). Finally, one study did not investigate stressors (Fox et al., 2024).

Across these studies, demographic reporting often lacks important information regarding race, ethnicity, religion, SOGIE identity and/or legal status. In sum, quantitative research on SOGIE refugees' mental health remains extremely limited, and studies examining post-migration stressors and their links to mental health are even more scarce – limited to two of the identified studies (Bleckmann et al., 2025; Fox et al., 2020).

Critical need for a measurement tool of post-migration stressors

Studies by Fox et al. (Fox et al., 2020) and Bleckmann et al. (Bleckmann et al., 2025) represent the only two identified attempts to quantitatively examine post-migration stressors and their association with mental health. Both draw on Minority Stress Theory (MST) (Brooks, 1981; Meyer, 2003), a well-established framework design to capture both external and internal psychological dimensions of stress.

These studies provide a solid foundation for growing quantitative research in SOGIE refugees' mental health (Bleckmann et al., 2025; Fox et al., 2020), demonstrating that both external (e.g., discrimination) and internal (e.g., heightened vigilance, internalized homophobia) dimensions of stress play a significant role in SOGIE refugees' mental health. For example, Bleckmann et al. demonstrates that both external and internal stressors contribute to mediating the association between refugee status and mental health outcomes and wellbeing (Bleckmann et al., 2025). Specifically, heightened vigilance predicted greater symptoms of depression, anxiety, and post-traumatic stress, internalized homophobia was associated with lower life satisfaction, and experiences of discrimination contributed to post-traumatic symptoms.

At the same time, a closer analysis reveals substantial differences between these studies and the original MST, in how stressors are implemented into measurable domains and then interpreted. For example, Bleckmann et al. evaluates some, but not all of the stressors dimensions identified by MST, missing the important factors of identity concealment and rejection expectation (explained in detail later) (Bleckmann et al., 2025). Work by Fox et al. is also based on MST but more loosely (Fox et al., 2020). While they evaluated identity concealment, they interpreted it in the opposite direction than the original framework, suggesting that revealing one's identity may

have damaging or negative effects on a person's stress due to increased stigma. While this is possible, MST specifically aims to disentangle internal from external dimensions of stress, and when isolated from the external consequence, revealing one's identity is known to be associated with positive outcomes (Grijak, 2017; Meyer, 2003). This demonstrates that the lack of a consistent measurement scale has been problematic for applying Minority Stress Theory to SOGIE refugee research, in particular for the dimensions of rejection expectation and identity concealment.

In sum, current evidence highlights a critical gap: although post-migration stressors are central to SOGIE refugees' mental health, they remain largely absent from quantitative research and, when examined, are measured inconsistently. Coupled with the more frequent and systematic – though still limited – quantitative literature on pre-migration trauma (Alessi et al., 2018; Bird et al., 2022; Hopkinson et al., 2017; Lasowski et al., 2023; Piwowarczyk et al., 2017), this creates an imbalance that risks undermining SOGIE refugees' wellbeing in two ways. Directly, it risks diverting attention away from structural barriers and ongoing post-migration stressors that could otherwise be addressed (Danisi et al., 2020; Gottvall et al., 2025). Indirectly, it risks reinforcing the “Global North protector” versus “Global South oppressor” narrative that, as discussed, has pervasive consequences across many aspects of SOGIE refugees' lives (Greatrick, 2019; Hiller, 2022; Peltonen & Jungar, 2018; Shakhshari, 2020; Tschalaer, 2020). As such, a comprehensive conceptual model and measurement scale provide the essential steps to improve genuine understanding of SOGIE refugees' experiences.

Theoretical perspectives

Alongside revealing key themes for SOGIE refugees' mental health and stressors, critical review of the literature also provided theoretical frameworks to ground these themes into a comprehensive conceptual framework. First, Intersectionality Theory and Cultural Clinical Psychology are broadly used as foundations to ensure a comprehensive consideration of the different aspects of SOGIE refugees' experience that may affect mental health. Specifically, Intersectionality theory guides us to consider how multiple minoritized identities of SOGIE refugees interact to shape their experience (Crenshaw, 1989; Nair et al., 2020; Purdie-Vaughns & Eibach, 2008), while Cultural Clinical Psychology provides foundations to account for factors across levels spanning from biological to sociocultural to systemic (Chentsova-Dutton & Ryder, 2025; Kirmayer & Ryder, 2016; Ryder et al., 2011, 2019). Then, Minority Stress Theory is directly used to identify the internal and external stressors and social factors mediating the link between life experiences and mental health outcomes (Brooks, 1981; Meyer, 2003). Finally, Branscombe's taxonomy is used to expand on these internal psychological stressors to capture nuanced aspects of identity threat experiences (Branscombe et al., 1999).

Minority Stress Theory

Minority Stress Theory (MST) (Brooks, 1981; Meyer, 2003) emerged in the years following the depathologization of homosexuality in the United States as a framework for explaining elevated rates of psychological distress among LGB populations as consequences of social stigma rather than intrinsic pathology. MST distinguishes between different types of stressors, as well as some protective processes, that contribute to mental health outcomes in minoritized groups. It has since been extended beyond LGB communities and is frequently applied in research on SOGIE refugees (Alessi & Kahn, 2017; Burgess et al., 2021; Fox et al., 2025; Golembe et al., 2020; Murugan et al., 2024; Piwowarczyk et al., 2017) – including the two aforementioned quantitative studies focused on post-migration stressors (Bleckmann et al., 2025; Fox et al., 2020).

MST identifies three types of stressors: *General stressors* are those caused by reasons beyond minority status – e.g., job loss due to economic recession or the death of a loved one. *Distal stressors* are external environmental

circumstances linked to minority status – including prejudice events like discrimination and violence. *Proximal stressors* are internal psychological experiences linked to minority status – including 1) expectations of social rejection and discrimination and consequent vigilance during social interactions, 2) concealment of one's identity for fear of harm, and 3) internalized homophobia or other stigma (Meyer, 2003). Meyer explains that the first proximal stressor – the expectation of rejection or discrimination – incorporates Branscombe's taxonomy of identity threats, as discussed in the next section (Branscombe et al., 1999). Stressors collectively shape mental health outcomes, together with other mediating factors including personal resilience and coping capabilities, and social support by both other individuals as well as community and associations (Meyer, 2003).

Rejection expectation & Branscombe's taxonomy of identity threat

Rejection expectation refers to the process by which marginalized individuals learn to expect rejection in their interactions with others, especially with members of the dominant culture (Meyer, 2003). Rejection does not have to be overt: people can typically perceive (e.g., via microaggressions) that their interlocutor does not fully accept them as “equal” but rather holds stigmatized views. A key mechanism by which stigma-based rejection affects mental health is the creation of an internal conflict between how we perceive ourselves and how others perceive us based on membership with a social group (Jones et al., 1984). Such conflicts can pose threats to the individual's self-concept and social identity (Branscombe et al., 1999; Jones et al., 1984). Meyer incorporates Branscombe's taxonomy (based on Social Identity Theory) of four kinds of identity threats, associated with specific conflicts that appear to closely align with SOGIE refugees' experiences (Branscombe et al., 1999; Hornsey & Hogg, 2000; Tajfel & Turner, 1979):

1. “Distinctiveness threat” occurs when the uniqueness of a person's social identity is undermined, especially when its distinct existence and characteristics within a broader social group are overlooked – e.g., when a bisexual individual is treated as “just gay” within broader 2SLGBTQI+ group.
2. “Acceptance threat” occurs when a person's belonging and acceptance by a social group they identify with is undermined, especially when the (potential) rejection is due to another aspect of their identity – e.g., when a bisexual individual is rejected by broader 2SLGBTQI+ group for being in a heterosexual relationship.
3. “Categorization threat” occurs when a person is categorized and perceived through the lens of a particular social group against the person's will, especially when such categorization is irrelevant to the situation – e.g., when one's 2SLGBTQI+ status is brought up in a job interview.
4. “Threats to the value of social identity” occur when the value a person's social group is undermined, especially by negative stereotypes like the group being incompetent or lacking morals – e.g., when one is told that being 2SLGBTQI+ is immoral.

Intersectionality Theory and intersectional invisibility

While research frameworks and hence terminology have evolved, distinctiveness and acceptance threats capture the still central issue of intersectional invisibility (Crenshaw, 1989; Nair et al., 2020; Purdie-Vaughns & Eibach, 2008). As social groups are typically defined around a single dimension of identity (i.e., being “LGBTQ” *or* “a refugee”), they implicitly adopt ethno-, cis-hetero-, and neuro-normative prototypes for the other dimensions (i.e., “white native non-Muslim LGBTQ” and “cisgender heterosexual refugee”). Based on this narrow definition, people with multiple marginalized identities belong to a social group based on a singular aspect of their identity, while not otherwise conforming with the remaining prototypical traits of the group (i.e., those of the more privileged members of that group). The outcome of this is twofold: First, the unique experiences and perspectives of people with intersectional identities remain overlooked or misrepresented. Second, individuals are assumed or

pressured to conform to the traits of more prototypical and privileged members of that group, effectively prioritizing one dimension of their identity over the other(s).

A large body of SOGIE refugee literature draws on Intersectionality Theory (Attia et al., 2025; Diab, 2024, 2025; Koko et al., 2018; Lee & Brotman, 2013; Skinta & Nakamura, 2021). Likewise, this theory provides a foundation for capturing the nuanced role of overlapping systems of oppression to the experience of stress in SOGIE refugees in our model.

Clinical Cultural Psychology approaches

Cultural Clinical Psychology emphasize the importance of considering the person-in-context as the core unit of analysis when conceptualizing mental health (Kirmayer & Ryder, 2016; Ryder et al., 2011, 2019). As such, they warn against the risk of not considering the role of cultural norms, social expectations, past and present experience and other contextual factors when developing models of mental health. As discussed later, culture and context can impact not only the stressors but also directly symptom presentation, as well as treatment.

In particular, Cultural Clinical Psychology frameworks, while focused on explaining heterogeneity in the symptom presentation of internalizing disorders, also provide a solid foundation to also understand the stressors contributing to such mental health outcomes (Chentsova-Dutton & Ryder, 2025; Kirmayer & Ryder, 2016; Ryder et al., 2011, 2019). The frameworks encompass factors including biological factors, personal history, immediate social interactions and context, cultural norms and influences, and systemic or ecosocial setting, emphasizing their role in the conceptualization of mental health. In the context of SOGIE refugees, this highlights the importance of accounting for stress deriving not only from trauma exposure, but also from factors including personal history of rejection, disrupted social relationships, cultural narratives about sexuality and gender, immigration and asylum systems that constrain autonomy and belonging.

Model of post-migration stressors in SOGIE refugees

Conceptual model

Figure 1 depicts a conceptual model of post-migration stressors in SOGIE refugees. The model is built to encompass the SOGIE refugee themes identified through literature review, and it is conceptualized drawing on the presented theoretical perspectives.

On one hand, discrimination and violence experienced in the country of origin lead to the proximal stressor of internalized homophobia, and the distal stressor of premigration trauma (while distinct, for image simplicity I include in-transit distal stressors in the same box). It also contributes to the intermediate factor of lack of genuine acceptance by the home community post-migration, which in turn contributes to the proximal stressors of rejection expectation and identity concealment, and the mediating factor of social isolation.

On the other hand, homonationalist narratives and assumptions on SOGIE refugees – from Western-based norms for SOGIE identities to negative discourse regarding religion and country of origin – contribute to the distal stressors of discrimination in the host country, and stressors associated with the asylum process (which are also due to other factors such as related to policies not depicted here). They also contribute to the lack of genuine acceptance of SOGIE refugees by local SOGIE communities, spaces and individuals – which in turn contributes to the same stressors and factors as the non-acceptance by the home community: social isolation, rejection expectation, and identity concealment.

Homonationalist narratives and assumptions additionally contribute to the widespread cultural insensitivity in mental health and general healthcare. The latter, together with systemic barriers and gaps in research hindering knowledge and advancement, contribute to the mediating factor of inadequate healthcare access and treatment, as well as insufficient access to resources and the associated resettlement stressors.

Ultimately, this model provides three distal stressors, three proximal stressors, and two mediating factors contributing to mental health outcomes. In the next section, I provide explanations and deeper understanding of these stressors based on Minority Stress Theory (Brooks, 1981; Meyer, 2003) and Branscombe's taxonomy of identity threats (Branscombe et al., 1999).

Developing the scale domains

The model domains are operationalized according to MST, as the processes explained by Meyer closely align with the key themes that emerged from qualitative interviews with SOGIE refugees (Meyer, 2003). Specifically, I propose model domains for SOGIE refugee's stressors that incorporate Branscombe's taxonomy of identity threats (Branscombe et al., 1999) into MST to better characterize rejection expectation. Together, these theories provide clear guidance on how to capture both *what happens externally* (distal stressors) and *how that makes one feel internally* (proximal stressor).

In order to support future applications that also span quantitative research, I will present the scale domains following the first step of Boateng's Best Practices for Developing and Validating Scales (Boateng et al., 2018). The proposed model includes the following domains:

1. Pre-migration and in-transit discrimination and violence (distal)
 - a. Subdomain of pre-migration
 - b. Subdomain of in-transit
2. Asylum process stressors (distal)
 - a. Subdomain of discrimination or re-traumatizing experiences
 - b. Subdomain of pressures to conform or conceal one's identity
 - c. Subdomain of uncertainty related to asylum decision
 - d. Subdomain of access barriers for legal services
3. Post-migration discrimination (distal)
4. Rejection expectation resulting in vigilance (proximal)
 - a. Subdomain of "Categorization threat"
 - b. Subdomain of "Value threat"
 - c. Subdomain of "Distinctiveness threat"
 - d. Subdomain of "Acceptance threat"
5. Identity concealment (proximal)
6. Internalized homophobia (proximal)
7. Social isolation (mediating)
8. Inadequate healthcare and services (mediating)
 - a. Subdomain of healthcare access barriers
 - b. Subdomain of inadequate or culturally insensitive care
 - c. Subdomain of access to other resettlement services (employment, housing...)

For most of these domains, I refer the reader to existing work containing already implemented measurements scales and/or detailed descriptions, as to promote consistency across studies:

1. Pre-migration trauma measures by Bird et al. and Hopkinson et al. (Bird et al., 2022; Hopkinson et al., 2017); the same measures that can be adapted to account for in-transit trauma with the aid of Yarwood et al. review (Yarwood et al., 2022).
2. Asylum process analyses by Danisi et al., among many others, offer a thorough understanding and robust basis to implement these measures (Danisi et al., 2020; Danisi & Ferreira, 2022)
3. Post-migration discrimination measures are provided by studies by Bleckmann et al. (Bleckmann et al., 2025) and Fox et al. (Fox et al., 2020) within a model similar to this one
4. *Rejection expectation resulting in vigilance* is explained in detail in the next section
5. *Identity concealment* is explained in detail in the next section
6. Internalized homophobia measure is provided by Bleckmann et al. (Bleckmann et al., 2025) within a model similar to this one
7. Social isolation measurements are provided by Fox et al. (Fox et al., 2020) and Gottlieb et al. (Gottlieb et al., 2020)
8. Healthcare access and utilization measures are provided by Gottlieb et al. (Gottlieb et al., 2020) and Ghabrial et al. (Ghabrial et al., 2024). Barriers to housing are analysed by McDowell & Collins (McDowell & Collins, 2024), and social capital is analysed by Karimi 2020 (Karimi, 2020). Social and organizational support and employment are analysed by Leyva-Moral et al. (Leyva-Moral et al., 2025).

Given their inconsistent implementation, the remainder of this section provides a clarification on the domain of “Identity concealment”, followed by a thorough explanation of the less understood domain of “Rejection expectation”. I break down this domain according to Branscombe’s taxonomy of social identity threat, and explain each of the resulting sub-dimensions using illustrative examples from empirical research.

Identity concealment

Among the proximal processes identified by Meyer (2003), identity concealment is especially relevant. Concealment is conceptualized as a coping strategy intended to avoid stigma and discrimination, yet it results in significant psychological distress through both cognitive and emotional channels. These include the constant cognitive burden of monitoring one’s behavior, the anxiety associated with possible exposure, conflicts in self-integrity, and internalization of negative social messages about one’s identity (Meyer, 2003). Concealment also restricts access to social support within the LGBTQ+ community, which further limits opportunities for resilience. Consistent with the broader literature, disclosure and authenticity are associated with better mental health outcomes.

Rejection expectation & identity threats

The four kinds of identity threats presented by Branscombe et al. (Branscombe et al., 1999; Hornsey & Hogg, 2000; Tajfel & Turner, 1979) are associated with specific conflicts that appear to closely align with SOGIE refugees’ experiences.

Categorization threat occurs when SOGIE refugees are categorized, perceived, and treated differently from other individuals against their will because of any facet of their identity. For example, it occurs when they are treated differently from native individuals within SOGIE spaces, where only their shared SOGIE identity should matter (Alessi et al., 2020; Danisi et al., 2020; Held, 2023). This includes occasions when comments are disguised as positive but are othering, such as reports of local SOGIE individuals commenting “I love Black people, you know?” without a reason (Chossière, 2021).

Threats to the value of one’s social identity occur when “treated differently” has negative connotations, such as when SOGIE refugees face negative stereotypes and moral narratives regarding their religion, country of origin

or other aspects of their identity. For example, this occurs through experiences of “you are Muslim; we don’t want you” that many refugees receive from the local SOGIE community (Alessi et al., 2020). Both categorization and value threats can manifest when SOGIE refugees face discrimination based on refugee status, SOGIE identity, religion, or other group memberships in contexts like housing, employment, or dating where only personal characteristics should matter (Alessi et al., 2020; Danisi et al., 2020).

Distinctiveness threat occurs when unique intersectional identities of SOGIE refugees are ignored, and characteristics, norms, and needs of broader groups are assumed – such when Western norms of SOGIE identities are applied across several aspects of migration and life (Danisi et al., 2020; Selim et al., 2023). Instead, acceptance threat occurs when SOGIE refugees are rejected from a social group or category that they actually belong to – such as when their SOGIE identity is not believed during asylum or when they are not accepted by local 2SLGBTQI+ communities (Casteleyn, 2024; Held, 2023).

Both acceptance and distinctiveness threats may occur when SOGIE refugees attend SOGIE spaces or meet SOGIE people in the host country (Alessi et al., 2020, 2023; Capous-Desyllas & Akkouris, 2022; Danisi et al., 2020; Fobear, 2016; Haghir-Vijeh & Clark, 2022; Held, 2023; Lee & Brotman, 2011) – as the experience of “not fitting in” may be associated with either threat. Acceptance threat may feel like “I want to belong but they tell me I do not fit in”; for example, when Muslim SOGIE refugees are directly rejected for their religion (Alessi et al., 2020; Held, 2023). Distinctiveness threat may feel like “they tell me I am just like them, but I feel that I have differences that make it harder to fully belong and they cannot see that”; for example, when local SOGIE individuals or even therapists act as if they fully understand the SOGIE refugee experience (Alessi et al., 2023; Danisi et al., 2020; Lee & Brotman, 2011).

These threats can also occur implicitly, e.g. when speaking a different language from the group as this can impact feelings of cultural belonging, with stress ameliorated by having interconnected social groups in the new language (M. M. Doucerain et al., 2015, 2023). Finally, both acceptance and distinctiveness threats may occur when SOGIE refugees are pressured to conform to western norms and stereotypes on 2SLGBTQI+ identity, which occurs especially during the asylum process (Dhoest, 2019; Gordon-Orr, 2021; Rehaag, 2009; Selim et al., 2023; Tschalaer, 2021).

Future directions

Developing an assessment scale of psychosocial factors for wellbeing

While the proposed model provides a theoretical foundation to understand post-migration distal and proximal stressors in SOGIE refugees, future research is needed to develop a validated scale for quantitative measurement of the proposed dimensions. In particular, I recommend that the scale be developed according to Boateng’s Best Practices for Developing and Validating Scales (Boateng et al., 2018). The article provides close guidance on each step involved in developing a new measurement scale for health, social, and behavioral research, with the goal of ensuring reliability and validity. The current model accounts for “Domain identification”, which is Part 1 of Step 1. Future work is needed to complete Part 2 of Step 1, “Item generation”, where specific questions are crafted. Beyond this, the process involves a total of 9 steps, including preliminary testing, and culminating in tests of reliability and validity.

An important consideration when developing this scale is that – as highlighted by the examples in the previous section – SOGIE refugee experiences often span multiple stressors categories, both distal and proximal. For example, housing discrimination on the basis of being a refugee, SOGIE, or of religion, spans the distal dimension of “discrimination” and the proximal dimensions of “categorization threat” and “threats to the value of social

identity”. On the opposite end, MST paired with the taxonomy for identity threat allows to capture and distinguish between external experiences that appear relatively similar at the surface, but for which subtle details make a big difference on the internal psychological effects they generate, such as distinctiveness versus acceptance threats. As such, it will be important during the “Item generation” step to craft questions that allow participants to report the multiple stresses emerging from the same experience.

Ultimately, following these steps will provide the field with the strong quantitative methodological foundation needed to address the research gap and call for quantitative studies in SOGIE refugee recently advanced by Rainbow Railroad (Noralla, 2025).

Developing culturally-sensitive diagnostic tools and care for mental health

While the premise of this article is based on poor mental health outcomes in SOGIE refugees as measured by DSM-V or other Western-based assessment tools, careful consideration should be given to how symptoms assessment in culturally diverse individuals. In particular, Chentsova-Dutton and Ryder provide a framework to explain symptom heterogeneity and how they shape clinical diagnosis and treatment (Chentsova-Dutton & Ryder, 2025). They provide a conceptual understanding of the different factors and levels that can modify symptom presentation, demonstrating that this can occur even without modifying internal dimensions for mental health – for example, by training individuals on what is the most culturally-appropriate way to express distress (Chentsova-Dutton & Ryder, 2020).

While there is consensus regarding the magnitude of mental health struggle in SOGIE refugees, it is possible that the specific diagnosis may not always be accurate. In fact, the diverse cultural background and the pressures to conform to Western norms are both factors that can strongly influence the way SOGIE refugee present mental health symptoms, even beyond shaping underlying stressors (Chentsova-Dutton & Ryder, 2020). As the asylum process is often thought to “train” individuals to emphasize SOGIE-based persecution (second to Western-norms-based SOGIE narrative) above all other stressors (Casteleyn, 2024; Selim, Helenelund, et al., 2025; Selim, Lindblad, et al., 2025), it is possible that other diagnoses such as depression may remain masked and untreated. Future work should leverage Cultural Clinical Psychology foundations (Kirmayer & Ryder, 2016; Ryder et al., 2019) and framework (Chentsova-Dutton & Ryder, 2025) to develop diagnostic tools that can capture the unique context of SOGIE refugees.

Ultimately, the literature review presented here highlights the importance of developing culturally-sensitive mental health care interventions. Work by Kahn et al. (Kahn et al., 2018), Alessi et al. (Alessi et al., 2023; Alessi & Kahn, 2017) and Reading and Rubin (Reading & Rubin, 2011) provides insights into the needs of SOGIE refugees and guidelines for providers. Important factors range from the cultural competency of providers to addressing stigma to promoting resilience. Across these and other studies, a central theme appears to be that support by SOGIE refugees communities – or similar – offer a much needed sense of belonging with positive outcomes on wellbeing (Kahn et al., 2018; Logie et al., 2016; Pelters et al., 2022; Reading & Rubin, 2011). Ultimately, this work provides the first step to address mental health needs in this population.

Limitations

While the model domains are grounded in empirical research and account for key post-migration stressors faced by SOGIE refugees, I recognize the inevitably limited scope of the proposed model. This article does not provide a comprehensive review of all relevant literature, nor accounts for all theoretical models relevant to this work.

In particular, additional prominent theories and frameworks may help expand on this basis. Above all, the framework from Burgess et al. – developed to understand suicidality in SOGIE refugees – provides a strong foundation for this by integrating MST, cumulative disadvantage and queer migration theories (Burgess et al., 2021). This work provides a great comprehensive understanding on mental health, including both pre-migration and post-migration factors, as well as both risk and protective factors. In the current review, I chose to draw on MST and Branscombe's taxonomy because they are more readily translatable to measurement tools, with several related scales having previously been developed (Balsam et al., 2011, 2013; Ma et al., 2024; Pala et al., 2017). Yet, I strongly support the addition of protective factors by Burgess et al. work, as these are not only an important part of the theoretical underpinnings, but also provide a base for practice implementation. Future work should incorporate this framework into a comprehensive understanding of both stressors and protective factors to SOGIE refugee mental health. Together with other applications of queer migration theory to SOGIE refugees work (Diab, 2025; Lunau & Andreassen, 2023), this will help not only with understanding stressors but also with reframing narratives around SOGIE forced-migration.

Moreover, future work should account for intersectional identities beyond being a refugee and a SOGIE individual. Themes from qualitative work in SOGIE refugees suggest that religion may be a particularly important dimension to consider. While use in migrants or refugees research remains limited, work with SOGIE Muslim individuals has drawn from Cognitive Dissonance Theory (Anderton et al., 2011; Festinger, 1957) and Transformative Intersectional Psychology (Etengoff & Rodriguez, 2021). Race is another aspect of SOGIE refugees' identities highlighted by qualitative work, and for which Critical Race Theory (Delgado & Stefancic, 2001) and its applications in SOGIE refugee work (Attia et al., 2025; Hazeldean, 2011; Lee & Brotman, 2013) may provide an ideal foundation. Ultimately, theories focused on specific identity intersectionality may best account for the unique experiences of individuals living at those intersections. Future work should draw from these and additional theories and frameworks, incorporating relevant domains into the proposed SOGIE refugee stressors scale, while also expanding beyond stressors.

Conclusions

This article provides a critical review of current research on SOGIE refugee's mental health. After highlighting the need for standard scales that can capture the breath of stressors across the stages of migration, a model for these stressors was proposed. By integrating Minority Stress Theory with the taxonomy of Social Identity Threat, the model hopes to adequately capture both distal and proximal stressors, addressing the oversimplification of identity-related dimensions seen in previous work.

The proposed model is only a beginning step in the broader efforts to understand SOGIE refugee wellbeing, and Future Directions are provided to advance this effort and support quantitative research that is comprehensive and culturally-grounded. First, I provided recommendations for developing an assessment tool according to Boateng's Best Practices for Developing and Validating Scales (Boateng et al., 2018). Second, I proposed Cultural Clinical Psychology frameworks to developed improved measures of mental health (Chentsova-Dutton & Ryder, 2025).

As each intersection of identities is unique, scales focused on capturing other parts of SOGIE refugee's identity should also be developed. Beyond generalized frameworks, future research should explore theories that capture specific intersectionality – considering aspects including religion, race, ethnicity, marginalized SOGIE identities like bisexual, transgender, non-binary, asexual, and lesbian, and understudied units like children, families or couples, among many others. Ultimately, by supporting quantitative research, this work will help advance a comprehensive understanding of SOGIE refugees' experiences, providing solid foundations to address their mental health needs.

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Table 1. Analysis of quantitative studies.

study	sample	identity and status	mental health assessment	contributing stressors	health-stressors association
Alessi et al., 2018	38 SOGIE refugees; Austria & Netherlands	SOGIE: gay (24), transgender female (5), lesbian (3), bisexual (3), queer or gender nonconforming (2), transgender male (1) Nationality: Syria (10), Iran (7), Iraq (5), Lebanon (4), Egypt (3), Pakistan (3), Jordan (1), Tajikistan (1), Chechnya-Russia (1), Palestine (1), Somalia (1) Religion: Muslim (34), Christian or Druze (4) (race/ethnicity not reported) Status: granted asylum (20), asylum case pending (15), temporary status granted (1), asylum denied (2)	QT DSM-V diagnostic tool PTSD: Posttraumatic Stress Disorder Checklist (PCL-5)	QL semistructured interview Questions: (a) “What was it like to be LGBTQ in your country of origin?” (b) “What factors motivated you to flee?” (c) “Tell us about any experiences of violence or discrimination during the migration process,” (d) “What was it like for you when you first arrived in Austria or the Netherlands?” Post-migration: minor	None
Bird et al., 2022	959 refugees (SOGIE & non-SOGIE); USA	SOGIE: persecution due to same-sex-behavior (107), other reason (852) Country of persecution: Uganda (557), other country criminalizing same-sex-acts (199), country non criminalizing same-sex-acts (203) Religion & race/ethnicity: not reported Status: asylum seeker (724), asylee (74), legal resident/green card (40), other (121)	QT screening tool PTSD: Short Screening Scale	QL narrative statements coded via content analysis method Narrative prompt: “Can you tell me what happened?” in direct relation to the persecution experience or experiences that led the client to seek asylum Post-migration: no	QT multiple linear regression Outcome: number of PTSD symptoms. Main predictor: persecution based on same-sex-act versus other reasons. Covariates: persecution, immigration status, gender, age, and total number of traumatic events
Bleckmann et al., 2024	409 SOGIE individuals (refugees & locals); Germany	Sex/gender V1: male (47.6%), female (42.8%), non-binary (6.9%), other gender identity (2.4%); Sex/gender V2: cisgender (92.2%), trans women (3.3%), trans men (4.5%); SO: gay (67.7%), bisexual (30.6%), heterosexual (1.4%) Nationality: 262 Germans; 139 Refugees with nationality: Syria (20.4%), Iran (27.6%), Iraq (8.6%), Afghanistan (4.6%), Other [<4% each] (38.8%) Religion & race/ethnicity: not reported Status: Asylum status not reported	QT screening tools & wellbeing measures PTSD: Primary Care Post-Traumatic Stress Disorder Screener (PC-PTSD-5) Depression/anxiety: four-item Patient Health Questionnaire (PHQ-4). Self-esteem: Rosenberg Self-Esteem Scale Life satisfaction: Satisfaction with Life Scale	QT Likert-scale measures Distal Minority Stressor discrimination: 9-Item Everyday Discrimination Scale Proximal Minority Stressors heightened vigilance: Heightened Vigilance Scale internalized homophobia: 9-Item Internalized Homophobia Scale Post-migration: yes	QT comprehensive path analysis Outcome: mental health/well-being. Predictor: minority status (LGBTQ+ refugees vs. LGBTQ+ Germans). Mediators: distal and proximal minority stressors
Fox et al., 2020	308 SOGIE forced migrants; USA, Canada	GI: cis-male (69.8%), cis-female (18.2%), genderqueer/genderfluid/other (7.5%), transgender female (3.6%), transgender male (1.0%); SO: gay (66.6%), lesbian (15.3%), bisexual (13.0%), heterosexual (3.2%), other (1.9%) Region of origin: Europe/Central Asia (58.9%), Caribbean (13.3%), Latin America (12.9%), Sub-Saharan Africa (9.5%), Middle East/North Africa (2.6%), Asia (2.3%); Top 5 Countries: Russia (36.7%), Jamaica (8.8%), Uganda (4.5%), Nigeria (4.2%), Belarus (4.2%) Religion & race/ethnicity: not reported Status: work permit received (44.8%); application: granted (34.7%), not submitted (13.0%), submitted (6.5%), denied (1.0%)	QT screening tools Cross-diagnostic mental health: Refugee Health Screener (RHS-15)	QT various assessments Social isolation: 4-item NIH Toolbox Loneliness measure & 4-item Patient-Reported Outcomes Measurement Information System (PROMIS) Short Form v2.0 Emotional Support scale Identity concealment: Outness Inventory Scale (modified to include gender) Community and social support: questionnaire with entries 1) Have & 2) Want More LGBTQ Friends, 3) Have & 4) Want More LGBTQ Friends from Home Country or Culture, 5) Number of LGBTQ Friends, 6) Source of Social Support Post-migration: yes	QT logistic regression Outcome: screening positive on the RHS-15. Predictors: loneliness, emotional support, identity disclosure, age, years in host country; and binarized post-secondary education, English proficiency, granted asylum, transgender, cis-female, bisexual status.
Fox et al., 2024	16 SOGIE forced migrants; USA, Canada	GI: cisgender men (3), cisgender woman (1), nonbinary or genderqueer (3), not collected (2); SO: gay (5), lesbian (1), queer (1), not collected (2) Country of birth: Guatemala, Jamaica, Nigeria, Russia, Saudi Arabia, Ukraine, and Venezuela. Religion & race/ethnicity: not reported Status: asylum seekers (16)	QT screening tools & wellbeing measures Cross-diagnostic mental health: Refugee Health Screener (RHS-15) Depression: ODSIS- Over- all Depression Severity and Impairment Scale Anxiety: OASIS—Overall Anxiety Severity and Impairment Scale PTSD: PCL- 5—PTSD Checklist for DSM-5 Loneliness: UCLA Loneliness Scale	None - Out of scope: study consists of pilot group therapy intervention. Included qualitative interview regarding intervention	N/A
Gottlieb et al., 2020	309 asylum-seekers; Germany	SOGIE: LGBTQI-specific shelter (32), other shelter (277) Sex/Gender. LGBTQI-specific shelter (of 27 known): female (30%), male (44%), other/diverse (26%); other shelter (of 244 known): female (34%), male (65%), other/diverse (1%) Status: LGBTQI-specific shelter (of 28 known): asylum-seekers (64%), granted (32%), rejected (4%); other shelters (of 232 known): asylum-seekers (35%), granted (47%), rejected (18%)	QT screening tools & wellbeing measures Depression: Patient Health Questionnaire-2 (PHQ-2) Anxiety: Generalized Anxiety Disorder-2 (GAD-2)	QT various assessments Quality of life: European Health Interview Survey - Quality of Life (EUROHIS-QoL) Social environment: three family status questions from European Health Interview Survey, plus “How many people are so close to you that you can count on them if you have a serious personal problem?” Subjective social status: 10-point MacArthur scale, asking to rank their social status separately for the country of origin and the host country, defining social status as based on education, money, and respected job Post-migration: yes	None - Out of scope: study focused on understanding factors for healthcare utilization
Hopkinson et al., 2017	61 SOGIE forced migrants; USA	SOGIE: Not collected Region of origin: Eastern Europe (26), West Africa (13), South America (7), Central Asia/Middle East (5), Central/North Africa (5), and the Caribbean (5). Religion: Christian or Muslim (82%), Jewish, Not Religious, or Other (18%); race/ethnicity not reported Status: undocumented (27), temporary visa (5), asylum application pending (21), asylee/refugee (4), not collected (4)	QT screening tools PTSD: Harvard Trauma Questionnaire (HTQ) Suicidal ideation: past & present, Y/N	QT coded interviews Sexual violence & persecution history (age, type, basis, identity of persecutors) Post-migration: No	QT Independent sample t-tests and chi-square analyses Outcomes: HTQ scores or suicidal ideation. Factors: history of sexual violence, age of first persecution, identity of persecutors

Table 1. *Continued.*

Study	sample	Identity and status	mental health assessment	contributing stressors	health-stressors association
Lasowski et al., 2023	66 SOGIE asylum seekers; USA (retro- spective chart analysis)	GIE: cisgender man (44), cisgender woman (10), transgender woman (9), transgender man (2), gender nonconforming (1); SO: gay (42), lesbian (9), straight (8), bisexual (3), chose not to specify (3), queer (1) Country of origin: Antigua and Barbuda (1), Bangladesh (1), Belarus (1), Burkina Faso (1), Colombia (5), El Salvador (5), Ghana (3), Guatemala (4), Guyana (1), Honduras (7), Jamaica (7), Kyrgyzstan (1), Lebanon (1), Mauritania (1), Mexico (9), Niger (1), Philippines (1), Russia (7), Saint Lucia (1), Senegal (1), Turkey (1), Uganda (3), Uzbekistan (1), Venezuela (2) Religion & race/ethnicity: Not reported Status: asylum seekers (16)	QT retrospective asylum process chart review: human rights program intake surveys, asylum seekers' sworn declarations, affidavits from forensic medical evaluations Psychological outcomes: PTSD, depression, anxiety, sleep disturbance, suicidality, self-harm, somatic or perceptual symptoms, substance use	QT retrospective asylum process chart review (same material) History of abuse Factors that may support health & wellbeing in the USA Post-migration: minor portion of Results	None
Mattelin et al., 2022	168,952 partici- pants in the Swe- dish Na- tional Public Health Survey (retro- spective analysis)	SOGIE & National origin: Western-born heterosexual (143,694), Western-born sexual- or gender minority (4,688), forced-migrant heterosexual (4,194), and forced-migrant sexual- or gender minority (253), other migrant heterosexual (4,300), other migrant sexual- or gender minority (285). Religion & race/ethnicity: Not reported Status: asylum seekers, persons in need of subsidiary protection, or refugees	QT Retrospective analysis of Swedish National Public Health Survey Cross-diagnostic mental health: General Health Questionnaire (GHQ-5) & Kessler-6 scale Mental wellbeing: Warwick Edinburgh Mental Well-Being Scale (WEMWBS) Suicidal ideation / attempts: Y/N Risk alcohol: Alcohol Use Disorder Identification Test (AUDIT-C) Risk gambling: Problem Gambling Severity Index (PGSI) Substance use: cannabis Y/N, other illicit drug Y/N	QT Retrospective analysis of Swedish National Public Health Survey Exposure to violence in the past 12 months, types (physical violence, threats, discrimination) Post-migration: Undefined (no information on country of violence)	None
Pi-wowarczyk et al., 2017	50 SOGIE refugees; USA (ret- rospec- tive chart analysis)	Sex: female (28), male (22); SO: lesbian (28), gay (18), bisexual (4) Region of origin: Africa (43), Asia (2), Europe (2), Central America (1), Middle East (1), South America (1); $\frac{3}{4}$ from Uganda Religion: Roman Catholic (19), Protestant (11), Muslim (8), Orthodox Christian (2), Hindu (1), Agnostic (1), Atheist (1), Other (1); Race/ethnicity not reported Status: asylum seeker (36), tourist visa* (6), student visa* (4), undocumented (3), refugee (1); *seeking asylum	QT retrospective chart review – center of National Consortium of Torture Treatment Programs. DSM-V diagnosis at intake: Anxiety disorder NOS, Panic disorder, PTSD, Major depression (single or partial remission, recurrent, psychotic features), Depressive disorder NOS, Chronic dysthymia, Alcohol abuse, Eating disorder	QT retrospective chart review (same center) Review of clinician notes: History of persecution: perpetrators, physical scars, types of persecution SOGIE identity concealment consequence of SOGIE disclosure Post-migration: No	Pearson's Chi square test of association, t-tests Outcome: PTSD. Factors: categories of perpetrators, physical scars, types of persecution, SOGIE concealment, consequence of disclosure

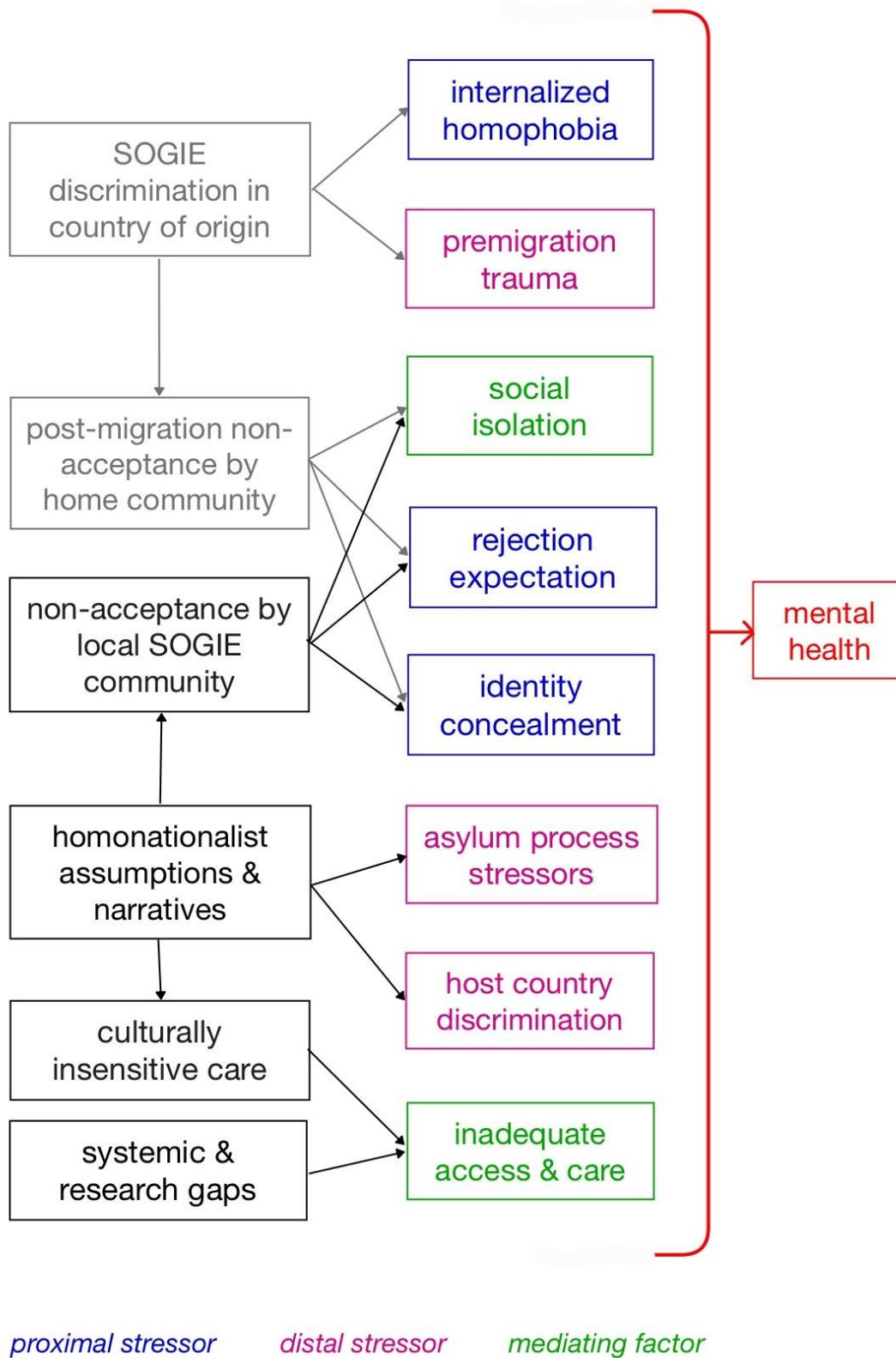


Figure 1. Conceptual model.